

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Methylphenidate (Concerta XL/Equasym XL) 18mg, 27mg, 36mg prolonged-release tablets - Monitoring and Ongoing Supply for the treatment of Attention Deficit Hyperactivity Disorder (ADHD) - Shared Care Monitoring

Summary of NICE / NICE CKS Guidelines for Attention Deficit Hyperactivity Disorder (ADHD) - Shared Care Monitoring

Overview

ADHD is a neurodevelopmental disorder characterised by persistent patterns of inattention and/or hyperactivity-impulsivity that interfere with functioning or development. Diagnosis must be made by a specialist psychiatrist or developmental paediatrician. Pharmacotherapy is part of a multimodal treatment approach and should be combined with psychological/behavioural interventions where appropriate.

Assessment for monitoring

- **Vital signs:** Blood pressure, heart rate, height, and weight must be measured at each visit. Monitor for cardiovascular risk.
- **Growth monitoring:** In children and adolescents, monitor height and weight regularly; plot on growth charts to detect growth faltering.
- **Psychiatric comorbidities:** Screen for mood disorders, anxiety, tics, substance misuse history.
- **Drug diversion risk:** Assess misuse potential; arrange shared care agreements and regular review.

Pharmacological management

- **Initiation:** Must be performed by a specialist (psychiatrist, developmental paediatrician, or specialist ADHD clinic).
- **Dose titration:** Specialist performs initial titration to effective and tolerated dose.
- **Ongoing monitoring:** Via shared care protocol with community pharmacy and primary care. Regular review of efficacy, tolerability, and adverse effects.
- **Treatment breaks:** Annual consideration of planned drug-free periods (e.g., school holidays) to monitor continued need and minimise long-term exposure.

Red flags requiring urgent specialist review

- New or worsening cardiovascular symptoms: chest pain, palpitations, dyspnoea, syncope.
- Significant psychiatric symptoms: new-onset psychosis, severe depression, suicidal ideation.
- Growth faltering in children/adolescents (significant deviation from growth trajectory).
- New or worsening tics.
- Evidence of medication diversion or misuse.
- Severe tolerance or loss of efficacy without dose escalation.

Patient Group Direction

for the supply/administration of

Methylphenidate 18mg, 27mg, 36mg prolonged-release tablets (Concerta XL/Equasym XL)

for the treatment of

Attention Deficit Hyperactivity Disorder (ADHD) - Shared Care Monitoring

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Continuation and ongoing monitoring of methylphenidate treatment for ADHD in patients whose treatment has been initiated and stabilised by a specialist under a shared care protocol. This PGD is for supply and monitoring only; initiation must be by a specialist.
Inclusion criteria	Adults aged 18 years and over with confirmed ADHD diagnosis established by a specialist psychiatrist or developmental paediatrician. Treatment has been initiated by the specialist and the patient is stable on a consistent dose for a minimum of 1 month. A formal shared care agreement is in place between specialist, community pharmacy, GP, and patient. Informed consent obtained; patient understands the nature of treatment, monitoring requirements, and Schedule 2 controlled drug status. Patient demonstrates capacity to consent; no evidence of substance

	misuse or significant psychiatric instability.
Exclusion criteria	ADHD diagnosis not yet confirmed by a specialist. Patient currently in titration phase (still being dose-adjusted by specialist). Severe cardiovascular disease: uncontrolled hypertension, heart failure, angina pectoris, structural cardiac abnormalities, cardiomyopathy, recent myocardial infarction, life-threatening arrhythmias. Concurrent use of monoamine oxidase inhibitors (MAOIs) or use within 14 days of stopping MAOI. Glaucoma. Phaeochromocytoma. Hyperthyroidism. Anorexia nervosa. Severe depression or suicidal ideation. Active psychosis. Pregnancy or breastfeeding.
Cautions	History of tics or Tourette's syndrome (may exacerbate tics; requires close monitoring). Seizure disorders or history of seizures (methylphenidate may lower seizure threshold). Hypertension or elevated cardiovascular risk factors. Bipolar disorder (risk of precipitation of manic episode). History of substance misuse or alcohol dependence. Hepatic impairment (mild to moderate). Renal impairment (mild to moderate). Growth monitoring required in children and adolescents; plot height and weight on centile charts. Prolonged use reassessment: at least annually consider planned treatment breaks (e.g., school holidays) to evaluate continued need and tolerance.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Methylphenidate 18mg, 27mg, 36mg prolonged-release tablets (Concerta XL or Equasym XL)
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Legal category	POM (Prescription Only Medicine), Schedule 2 Controlled Drug
Storage	Store below 25°C. Protect from moisture. Schedule 2 Controlled Drug. Must be stored securely, out of reach of children and others. Kept in original child-resistant container.
Route/method of administration	Oral. Swallow tablet whole with water, in the morning only. Take with or without food. Do not crush, chew, divide, or open capsules.
Dose and frequency	As prescribed by the specialist; usual range 18-54mg once daily in adults (BNF maximum 54mg/day). Concerta XL and Equasym XL are prolonged-release formulations given once daily in the morning. Swallow tablet whole with water with or without food. Do not crush, chew, or divide the tablet. Tablets contain a dose-delivery system; crushing may compromise the extended-release mechanism. Dose may be adjusted by specialist; pharmacy supplies at dose prescribed.
Quantity to be administered and/or supplied	Up to 28 tablets (28-day supply for once-daily dosing). Individual pack size determined by specialist prescription.
Maximum or minimum treatment period	Ongoing with structured monitoring. Clinical review by pharmacy at each supply visit (vital signs, side effects, efficacy). Formal specialist review at least annually. Consider planned treatment breaks (e.g., during school holidays or extended time off) to assess continued need and permit drug-free periods. Long-term use should be periodically reassessed; continuation is not automatic.
Adverse effects	Very common: Insomnia, headache, decreased appetite, nervousness/anxiety. Common: Nausea, dry mouth, abdominal pain, tachycardia, palpitations, mood changes (irritability, emotional lability, aggression), dizziness, tremor, weight loss. Uncommon: Psychosis, hallucinations, tics or worsening of existing tics, seizures. Rare: Sudden cardiac death (primarily in those with underlying structural cardiac defects; risk estimated at 1 per 1 million), stroke, myocardial infarction, arrhythmias, severe hypertension. Very rare: Priapism (prolonged painful erection).

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP

	• name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the methylphenidate formulation. Counsel on Schedule 2 controlled drug status, secure storage, and the importance of regular monitoring appointments.
Follow-up advice to be given to patient or carer	Take the tablet in the morning only; swallow whole with water. Do not crush, chew, or divide the tablet. This is a Schedule 2 controlled drug; store securely out of reach of others and children. Do not share with anyone. Attend all scheduled monitoring appointments; vital signs (blood pressure, heart rate) and weight will be checked at each visit. Report chest pain, palpitations, shortness of breath, or fainting immediately to your GP or emergency services. Report any mood changes (depression, aggression, anxiety), new-onset tics, or thoughts of self-harm to your healthcare team. Maintain regular contact with your specialist psychiatrist for at least annual review of your treatment. Inform all healthcare professionals (dentists, other doctors) that you are taking methylphenidate. Do not suddenly stop taking this medication; discuss any concerns about continuing treatment with your specialist. Planned breaks from medication (e.g., school holidays, weekends) may be discussed with your specialist to allow periodic drug-free periods. Be aware that methylphenidate may cause decreased appetite, insomnia, or mood changes; discuss these effects at your monitoring appointments.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017
<https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Attention deficit hyperactivity disorder
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Methylphenidate 18mg, 27mg, 36mg prolonged-release tablets (alternative formulations)

for the treatment of

Attention Deficit Hyperactivity Disorder (ADHD) - Shared Care Monitoring

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Continuation and ongoing monitoring of methylphenidate for ADHD under specialist-led shared care protocol. For monitoring and supply only; not for initiation.
Inclusion criteria	ADHD confirmed by specialist. Stable on methylphenidate for ≥ 1 month. Shared care agreement in place. Informed consent; capacity to consent confirmed. No evidence of substance misuse or acute psychiatric instability.
Exclusion criteria	ADHD not specialist-diagnosed. Currently in titration phase.

	Severe cardiovascular disease (see primary formulation above). Concurrent MAOIs or within 14 days. Glaucoma, phaeochromocytoma, hyperthyroidism. Anorexia nervosa. Severe depression/suicidal ideation. Active psychosis. Pregnancy/breastfeeding.
Cautions	Tics/Tourette's syndrome. Seizure disorders. Hypertension/cardiovascular risk. Bipolar disorder. Substance misuse history. Hepatic/renal impairment. Growth monitoring (children/adolescents). Annual reassessment of need; consider treatment breaks.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Methylphenidate 18mg, 27mg, 36mg prolonged-release tablets (generic or alternative brands)
Legal category	POM (Prescription Only Medicine), Schedule 2 Controlled Drug
Storage	Below 25°C, protect from moisture. Schedule 2 Controlled Drug - secure storage required.
Route/method of administration	Oral, prolonged-release tablets swallowed whole with water, morning only.
Dose and frequency	As prescribed by specialist; typical adult range 18-54mg once daily. Morning administration only, with or without food. Swallow whole; do not crush or divide.
Quantity to be administered and/or supplied	Up to 28 tablets (28-day supply).
Maximum or minimum treatment period	Ongoing with regular monitoring (each supply visit: BP, HR, weight). Formal specialist review at least annually. Planned treatment breaks to be considered.

Adverse effects	Very common: Insomnia, headache, appetite decrease, nervousness. Common: Nausea, dry mouth, abdominal pain, tachycardia, palpitations, mood changes, dizziness, tremor, weight loss. Uncommon: Psychosis, hallucinations, tics, seizures. Rare: Sudden cardiac death (in structural cardiac defects), stroke, MI, severe hypertension.
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Follow-up advice to be given to patient or carer	Take the tablet in the morning only; swallow whole with water. Do not crush, chew, or divide the tablet. This is a Schedule 2 controlled drug; store securely out of reach of others and children. Do not share with anyone. Attend all scheduled monitoring appointments; vital signs (blood pressure, heart rate) and weight will be checked at each visit. Report chest pain, palpitations, shortness of breath, or fainting immediately to your GP or emergency services. Report any mood changes (depression, aggression, anxiety), new-

	onset tics, or thoughts of self-harm to your healthcare team. Maintain regular contact with your specialist psychiatrist for at least annual review of your treatment. Inform all healthcare professionals (dentists, other doctors) that you are taking methylphenidate. Do not suddenly stop taking this medication; discuss any concerns about continuing treatment with your specialist. Planned breaks from medication (e.g., school holidays, weekends) may be discussed with your specialist to allow periodic drug-free periods. Be aware that methylphenidate may cause decreased appetite, insomnia, or mood changes; discuss these effects at your monitoring appointments.
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